Activity 6: Mental-Wellbeing Policy-to-Practice Playbook

Learning outcome: LO2 Objective: Translate policy into safe day-to-day manager and team practices
Expected output: Policy-to-practice manager playbook and implementation evidence plan Primary source: https://www.tal.sg/tafep/employment-practices/mental-health

Goal

Turn a generic mental-wellbeing policy into observable routines, decision rights, scripts and escalation paths.

Real-world scenario

Singapore's tripartite guidance encourages employers to strengthen mental wellbeing through organisation-, team- and individual-level practices, including leadership, supportive managers, work-life harmony, access to help and appropriate confidentiality.

EastLine Distribution, a fictional employer, has a two-page wellbeing policy but inconsistent practice. One manager demands clinical details, another ignores repeated overtime, and employees do not understand when HR or emergency support becomes involved.

Evidence boundary

Public case facts and fictional learner data must never be mixed. Every numerical or organisational training input is labelled as an assumption.

Training assumptions / guardrails

- All characters and events are fictional learning scenarios.
- Managers are not clinicians and must not diagnose or provide therapy.
- The organisation has HR, an EAP and a documented emergency escalation route.
- Work adjustments depend on role and risk but should use only necessary information.

Questions

1. Which policy statements need a concrete routine, tool or decision right?
2. What can managers ask, record, decide and escalate?
3. How should the playbook handle immediate safety concerns and ordinary work support differently?
4. What evidence demonstrates implementation beyond attendance at training?

Detailed activity procedure

Step 1: Extract policy intent

Highlight each promise, duty, boundary and escalation requirement. Rewrite vague verbs such as support or promote as observable outcomes.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 2: Map employee moments

Identify normal check-ins, visible change, request for help, work adjustment, conflict, absence, return-to-work and immediate safety concern.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 3: Assign decision rights

For every moment, state employee, manager, HR, safety and provider roles; identify what is outside manager authority.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 4: Write conversation guides

Create short Notice-Ask-Listen-Act-Follow-up prompts using observable facts, privacy-respecting questions and no diagnosis.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 5: Define documentation

Specify the minimum record, access, storage and retention. Separate operational adjustment information from clinical details.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 6: Build escalation routes

Distinguish routine referral, complex case consultation, urgent risk and emergency response. Include after-hours paths and handoff confirmation.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 7: Plan capability rollout

Use demonstration, practice scenarios, feedback and supervisor coaching. Add resources, communications and employee-facing expectations.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 8: Verify implementation

Select audits, employee feedback, manager confidence, response-time and case-quality evidence; assign corrective action and review.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Deliverable

Policy-to-practice manager playbook and implementation evidence plan

Acceptance criteria

The playbook covers routine through emergency situations, limits collection and diagnosis, names decision rights and handoffs, provides usable scripts and defines evidence of practice quality rather than training completion alone.

Reflection and debrief questions

1. Which assumption changed your recommendation most, and why?
2. What evidence would you request before relying on the result?

3. Which stakeholder owns the next decision or mitigation?
4. Which review trigger or future exercise should follow?

Source note

Source accessed 16 August 2026: https://www.tal.sg/tafep/employment-practices/mental-health